



TARGET POPULATION

ALL ADULTS ≥ 18 YEARS OF AGE

HIGHER RISK POPULATION

FROM ALCOHOL RELATED HEALTH EFFECTS*

- Pregnant, Children
- Breast feeding
- Liver disease
- Drivers (risks of injuries, intentional or unintentional, include falls, drowning, burns, sexual assault, intimate partner violence and suicide)
- Alcohol dependence syndrome

(Note: Every individual is at risk of alcohol dependence, but some are at a higher risk. Alcohol can amplify the effects of sedatives, causing respiratory depression and overdose.)

Ref: WHO, 2024

DIAGNOSIS

ALCOHOL USE DISORDERS IDENTIFICATION TEST (AUDIT)

Hazardous drinkers are those with AUDIT ≥ 5 for males and ≥ 4 for females.

ALCOHOL DEPENDENCE SYNDROME AND WITHDRAWAL

In patients dependent on alcohol, withdrawal symptoms appear 6-24 hours after the last drink.

- **Mild** - tremor, agitation, nausea
- **Moderate** - hallucinations
- **Severe** - confusion, disorientation, severe tremors, fast pulse, sweating, fever, dehydration.

Seizures can occur 12 to 48 hours after the last drink.

(Note: Not all patients who are misusing alcohol will have serious withdrawal symptoms.)

ALCOHOL IMPACT ON HEART

FEATURES OF DILATED CARDIOMYOPATHY INCLUDE:

- Difficulty breathing, increased when lying flat
- Ankle edema

SCAN THE QR CODE BELOW TO
PERFORM 10- ITEM AUDIT (ALCOHOL
USE DISORDERS IDENTIFICATION TEST)



THIS IS 1 UNIT



Half Pint (284 ml)
of regular Beer



1 Small glass
of Wine



1 single measure
(25 ml) of Vodka/Spirits

WHAT IS HARMFUL DRINKING ?



Self-reported consumption of ≥ 21 standard drink units (SDU) per week for males (equivalent to 210g ethanol)



Self-reported consumption of ≥ 14 standard drink units per week (equivalent to 140g ethanol) for females.

One SDU amounts to 10g of pure ethanol.

Raksi contains 25%, Jand or Chhang contains 12% and Tongba contains 5.5% alcohol.



ALCOHOL CONSUMPTION IS RESPONSIBLE FOR 2.6 MILLION DEATHS WORLDWIDE, REPRESENTING 4.7% OF ALL DEATHS.

Ref: WHO, 2019

OUR COLLABORATORS



Ref. PEN Manual (MoHP), 2075



Hôpitaux
Universitaires
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KHDC
NEPAL



HARMFUL ALCOHOL CONSUMPTION IDENTIFICATION GUIDE FOR PRIMARY CARE PROVIDERS

USE THIS GUIDE TO

- IDENTIFY AND REACH PATIENTS WITH ALCOHOL CONSUMPTION
- MANAGE AND REFER PATIENTS WHEN NEEDED

Harmful drinking refers to alcohol use that leads to significant negative health consequences, both physically and psychologically.

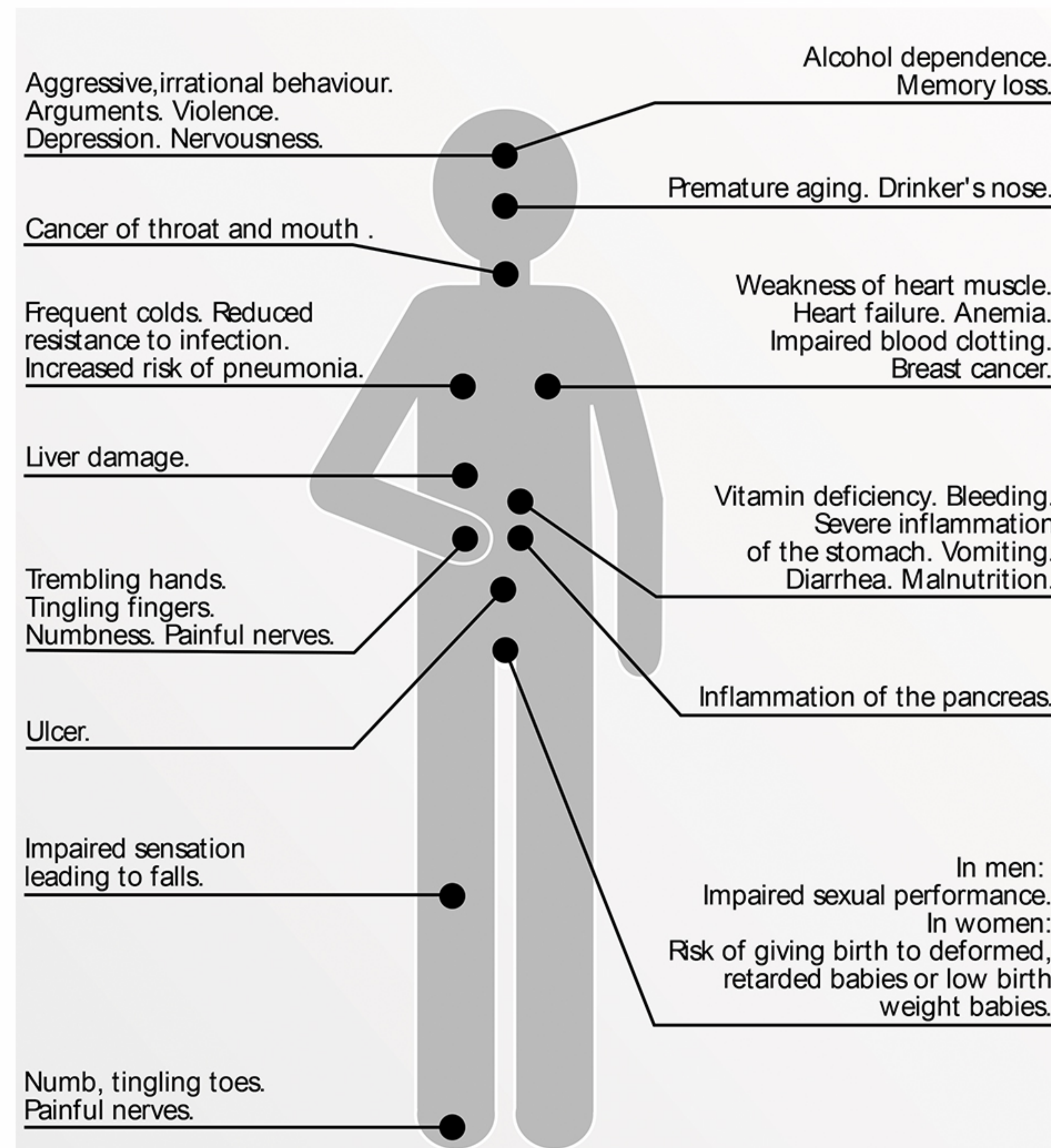


17.4% NEPALESE CONSUME ALCOHOL, IT IS THE PRIMARY CAUSE OF ROAD TRAFFIC ACCIDENT IN NEPAL

Ref: Step Survey, 2013



EFFECTS OF HARMFUL DRINKING



Ref: WHO, Alcohol brief intervention training manual for primary care, 2017



CLINICAL ASSESSMENT

Personal history: Number of days drinking in last 1 week; Quantity consumed per drinking day.

Family history: History of harmful alcohol drinking in the family

Past medical history: Seizures; tingling and numbness of extremities; liver disease; abdominal distension; heart disease

- History of withdrawal symptoms including seizure
- History of difficulty breathing or ankle edema

Clinical examination: Pallor, Jaundice, Edema, Ascites (Fluid in abdomen), Lung base crackles



INVESTIGATIONS (If available)

- CBC (MCV > 110),
- Liver function test (AST/ALT ratio: 2 : 1),
- Echocardiography

5 A'S OF ALCOHOL CESSATION

ASK

Identify alcohol user at every visit by asking in a friendly, non-accusatory way
If answer is **"Yes"** - perform WHO AUDIT

ADVISE

Persuade alcohol users that they need to quit in a clear and strong manner, considering traditional cultural beliefs in a personalized way

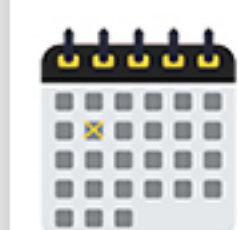
ASSESS

Ask 2 questions

Would you like to be someone who doesn't drink?	Yes	Unsure	No
Do you think you have a chance of not drinking?"	Yes	Unsure	No

ASSIST

Set a duration, Reach out for family and social support, Anticipate withdrawal



S SET A QUIT DATE



T TELL FAMILY, FRIENDS, COWORKERS



A ANTICIPATE CHALLENGES



R REMOVE ALCOHOL PRODUCTS

ARRANGE

Schedule follow-up every month to assess problems and need for expert consultation

5 R'S OF ALCOHOL CESSATION



RELEVANCE

Encourage the patient to indicate how quitting alcohol is personally relevant to him or her.



RISK

Encourage to identify negative consequences of alcohol



REWARDS

Encourage to identify potential benefits of alcohol cessation



ROADBLOCKS

Encourage to identify barriers to stopping alcohol consumption and how he or she could address them



REPETITION

Repeat assessment of readiness to quit on every visit to Health Center.

Would you like to be someone who doesn't drink?	Yes	Unsure	No
Do you think you have a chance of not drinking?"	Yes	Unsure	No



CRITERIA FOR REFERRAL

- Symptoms of withdrawal seizures, confusion, hallucinations or delirium
(Note: In case of severe tremors and withdrawal seizures seek telemedicine help and refer after stabilisation dose of diazepam as advised)
- Difficulty breathing or ankle swelling
(Note: For alcohol effects on the heart (Dilated Cardiomyopathy) Refer to KHDC leaflet on Heart Failure)
- Jaundice, abdominal distension
- Severe abdominal pain
- Vomiting of blood
- Frequent vomiting
- Malnourished
- **AUDIT score of >11**

SEEK TELEMEDICINE HELP FOR ANY CLINICAL QUERIES

CONTACT US

For any clinical queries / If anything in the document is difficult to understand



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